CASE STUDY

Designing a Role-Based Clinical Training Program

A learning strategy for a pain clinic workflow launch

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LEARNING NEEDS ASSESSMENT

Establishing a Baseline

I began this task by having an informal conversation with a PGY4 working in an outpatient pain clinic in Fort Lauderdale, Florida. The conversation focused on the clinic's transition to VBC and surfaced three main concerns with the current workflow (at right).

Additional Action Steps for the Learning Needs Assessment:

1. Shadow the workflow through the eyes of each role
2. Map where handoffs and information actually break down
3. Confirm gaps are behavioral, not a lack of knowledge

1 Inaccurate pre-charting

Copy-forward notes stop matching reality once a complaint changes, forcing documentation to be rebuilt from scratch.

2 Inconsistent handoffs

Residents and attendings sometimes evaluate the same patient independently, duplicating clinical work.

3 Billing and coding rework

10 to 12 charts corrected every few days for unsupported diagnoses and CPT/ICD-10 mismatches.

AUDIENCE & ROLE-BASED DESIGN

One workflow, four learning profiles

Training readiness by role across the skills the new workflow touches.

X = Essential to Role | Completes Independently | No Training Necessary | Needs Ongoing Support | Needs Training & Direction

Role	Intake Documentation	Handoff Communication	Coding & Compliance	Patient Communication	EMR & Technology
Front Desk	X	X		X	
Resident	X	X	X		
Therapy Staff		X			
Attending		X			X

INSTRUCTIONAL DESIGN APPROACH

Different needs, different methods

J

Job aids

Quick-reference intake fields, used in the moment.

S

Scenario practice

Residents rehearse rebuilding a pre-chart before it happens live.

C

Coaching & shadowing

Billing accuracy improves fastest through real-time chart feedback.

Why this mix: the gaps are behavioral, so we lean on practice and coaching over passive eLearning.

SAMPLE DELIVERABLE

Front Desk Intake Job Aid

Complete before sending the patient back for pre-charting.

- ☐ **Chief complaint (location of pain)**
  One sentence, in the patient's own words.
- ☐ **Acute vs. chronic**
  Check one.
- ☐ **Improved / Unchanged / Worsening**
  Check one, compared to the last visit.
- ☐ **Reason for today's visit**
  One sentence.

CLINICAL ACCURACY & STAKEHOLDER PARTNERSHIP

Engaging the clinical SMEs

MD

Attending physicians

Validate clinical scenarios and sign off on any content describing diagnosis or treatment logic.

$

Billing & coding team

Confirm CPT/ICD-10 examples reflect real compliance requirements, not simplified assumptions.

O

Operational leaders

Own rollout timing, staffing coverage, and reinforcement across locations.

T

Technology/LMS team

Ensure delivery, tracking, and EMR-adjacent tools work as designed before launch.

LEARNER EXPERIENCE

Immediate usability

1. **Practical** — Every module opens with the real friction point it fixes, not a policy statement.
2. **Inclusive** — Content is built and reviewed with input from front desk through attending, not one voice.
3. **Easy to apply** — Job aids live where the work happens; nothing requires memorizing a manual.
4. **Respectful of time** — Short, role-specific modules replace one long session everyone half-attends.

TECHNOLOGY & DELIVERY PLAN

Deliver through the LMS, design for reuse

1

Author once

Short modules and job aids built as reusable objects.

→

2

Host in the LMS

Assignment rules route each module to the right audience.

→

3

Track completion + application

Completion data paired with real workflow metrics.

→

4

Update on a cycle

Content revised whenever the workflow changes.

Adoption and KPIs

Reinforcement & Adoption

Manager-led huddles

Short check-ins reinforcing the handoff protocol in context.

EMR status indicator

An in-system flag showing the primary evaluator per patient.

Peer champions

One trained champion per role, available post-launch.

30/60/90 check-ins

Scheduled follow-ups to catch drift back to old habits.

Measurement & Success Metrics

↓

Copy-forward documentation errors

Chart audit, before vs. after rollout

↓

Billing correction volume

Baseline: 10–12 charts every few days

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Time to competency

Speed to independent, accurate execution

↑

Handoff clarity

Staff-reported confidence in who evaluates